



Medication: Olanzapine pamoate (Zyprexa Relprevv®) - Administration (Mental Health) Policy & Procedure

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to⁷:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Policy statement

- To ensure patients and staff are aware of the monitoring requirements before commencing on olanzapine pamoate¹ and are familiar with and able to carry out best practice in the administration of olanzapine pamoate.
- All aspects of care are in line with the [Chief Psychiatrist's Standards for Clinical Care](#).¹

Background

- Olanzapine pamoate is the long-acting injectable form of olanzapine and is indicated for antipsychotic maintenance treatment of schizophrenia in adults. There is an occurrence of Post Injection Syndrome (PIS) in approximately 0.07% of injections (1 in 1400), possibly due to inadvertent intravascular (IV) injection of a portion of the dose - the molecule is more soluble in blood than muscle. PIS presents with many symptoms consistent with oral olanzapine overdose including sedation, confusion and disorientation, speech and motor impairment and dizziness. PIS is not dose, frequency or time point specific, and the risk of occurrence exists following every administration. PIS has not been reported with other long acting antipsychotic injections.
- A post marketing study reported that the time to onset of the initial signs and symptoms of PIS occurred within the first hour after injection in 91% of cases, within 2 hours in 96% of cases and within 3 hours in 98% of cases. The remaining 2% of cases were identified beyond 3 hours. A pre-marketing study reported the time to incapacitation ranged from 10 minutes to 5 hours, with the median time to incapacitation being 35 minutes after the median time of symptom onset. This

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highlights the importance of the post monitoring period as it may be some time before a patient develops overt symptoms, which may interfere with their ability to seek assistance. Full recovery from PIS usually occurs within 24 - 72 hours.⁶

WARNING

The potential signs and symptoms of sedation and/or delirium consistent with olanzapine overdose exists after every injection of olanzapine pamoate monohydrate. Olanzapine pamoate monohydrate should be administered by appropriately qualified health professionals and patient monitored for at least two hours after each injection.

Definitions

Hypersensitivity	Extreme physical sensitivity to particular substances or conditions.
Long Acting Injection (LAI)	A pharmacological agent with a prolonged effect because of a formulation resulting in the slow release of the active principle or the continued absorption of small amounts of the dosage of the drug over an extended period.
Post Injection Syndrome (PIS)	Signs and symptoms following injection of olanzapine Zyprexa (Relprevv [®]) consistent with Post Injection Syndrome (PIS) include: • Sedation • Confusion / Disorientation • Impairment of Speech (Dysarthria) and Motor (Ataxia) • Dizziness • Agitation/Anxiety/Aggression • Extrapyramidal symptoms (EPSE)
Verbal Consent	Where a verbal (compared with written) agreement to a treatment or procedure is discussed with and agreed to by a patient.

Patient profile for treatment with olanzapine pamoate

- History of previous positive response to oral olanzapine.
- Non-compliance or adherence issues with oral medication.
- All other depot antipsychotic medications have been considered.
- Consents to the 2-hour observation period requirement after the administration of the depot. Consents to not travel alone to their post injection destination. Consents to not drive or operate machinery for the remainder of the day after the injection.
- Depending on the individual circumstances, transport assistance will be considered for patients who cannot be conveyed to and from the area where the injection is to be administered. This may take the form of a staff member assisting or the use of a taxi voucher.

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Prior to prescribing

- The relevant community team who will be following the patient has been involved in the decision-making process at an early stage.
- The medication is to be prescribed and administered by staff who have specific knowledge of symptoms that may indicate PIS.
- The patient has received verbal and written education on olanzapine pamoate including safety precautions with PIS and the need for the 2-hour observation after the administration of the depot and has agreed to this.
- The patient has agreed that they will not travel alone to their post-injection destination and are aware that they are unable to drive or operate machinery for the remainder of the day.
- The treating Consultant or Registrar/Medical Officer (MO)/Case Manager will liaise with the appropriate contact for the area where the patient is going to be managed for their injection (in-patient unit, community mental health centre) to arrange the date, time and place for the injection.

Training requirements and responsibilities for administration and post injection monitoring

- Nursing staff are responsible for administering olanzapine pamoate and have been trained in reconstitution, administration and risk minimisation requirements.
- Patients are to be advised of potential side effects each time the medication is prescribed or administered.² Any PaRK MHS clinician trained in recognizing the signs of olanzapine post injection syndrome may monitor patients after depot administration.

Starting dose

Olanzapine pamoate is to be initiated based on the oral daily equivalent dose of olanzapine as per table 1 below:

Table 1

Target oral olanzapine dose	Recommended starting dose of Zyprexa Relprevv	Maintenance dose after 2 months of Zyprexa Relprevv treatment
10mg/day	210mg/2 weeks or 405mg/4 weeks	150mg/2 weeks or 300mg/4 weeks
15mg/day	300mg/2 weeks	210mg/2 weeks or 405mg/4 weeks*
20mg/day	300mg/2 weeks	300mg/2 weeks*

*Doses greater than 405mg/4 weeks or 300mg/2 weeks have not been studied in clinical trials.

Reconstitution

- Standard aseptic technique is to be maintained when reconstituting the medication. Hand hygiene must be carried out as per the [RKPG Hand Hygiene \(Infection Control\) Policy](#)³

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- It is recommended that gloves are to be worn during the reconstitution process as olanzapine pamoate may be irritating to the skin. Flush with water if contact is made with the skin.¹
- Use only the supplied diluent for reconstitution.
- Each dose requires a different amount of diluent to be added and volume to be injected as per the table below, with all dosed reconstituting to a fixed concentration of 150mg/mL olanzapine pamoate.

Table 2

Dose	Zyprexa Relprevv vial strength (mg)	Volume of diluent to be added	Volume to be injected
150mg	210mg	1.3mL	1.0mL
210mg	210mg	1.3mL	1.4mL
300mg	300mg	1.8mL	2.0mL
405mg	405mg	2.3mL	2.7mL

- **Note:** there will always be left over diluent and solution in the vial.
- Once reconstituted, the injection solution should be used as soon as possible. The reconstituted product may be stored for up to 6 hours at room temperature (25°C) before being used.²

Administration

Prior to administration

- Confirm verbal patient consent to receive olanzapine pamoate and willingness to co-operate with the subsequent observation period. If the patient states they are unwilling to remain for the observation period, contact the treating Consultant or Registrar/MO for further advice.
- Confirm the patient will not be travelling to the post-injection destination alone and will not be driving or operating machinery for the remainder of the day.
- Assess that the patient is not intoxicated with drugs or alcohol. If the patient is intoxicated with drugs or alcohol, contact the treating Consultant or Registrar/MO for further advice.

Administration

- Olanzapine pamoate is recommended for deep gluteal injection only (not for subcutaneous or intravascular injection).⁴ Deltoid administration can occur with documented Consultant Psychiatrist approval only. This approval will be documented on the patient's medical record as well as on Psolis.
- The syringe is to be aspirated for several seconds prior to injecting – with any sign of blood the dose must be discarded, and a new dose reconstituted.
- Use the appropriate specified gauge safety needle for the patient's body weight that is provided by the manufacturer:

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- * In non-obese patients use 19 gauge 1.5 inch needle.
- * In obese patients use 19 gauge 2 inch needle.

Community Mental Health olanzapine pamoate observation process

- Administration of olanzapine depots are completed by a nursing staff member, who commences notation on the Olanzapine Observations checklist. Subsequent visual observations can be provided by the nursing staff member or any PaRK MHS clinician trained in recognizing the signs of olanzapine post injection syndrome
- Clinician to please ensure patient sticker is on the checklist and it is dated.
- Clinician to observe the patient for 2 hours, checking in with the patient every 30 minutes and note on record sheet.
- The Olanzapine Observations checklist should be filed under 'medications' in the patient's medical record.

Post injection

- Monitor patient every 30 minutes for 2 hours by assessing:
 - * Level of alertness.
 - * Orientation to person, time and place.
 - * Physical symptoms (dizziness, dysarthria, ataxia).
 - * Monitoring is to be done by clinicians who have specific knowledge of symptoms that may indicate PIS.
 - * Record the above on observation chart.
- **Note:** the common symptoms of PIS include sedation, confusion/disorientation, dysarthria, dizziness and ataxia.

Action if PIS suspected

- If the patient is in the community setting phone "000" for an ambulance for transfer to the Emergency Department (ED).
- If the patient is in the hospital setting and meets the requirement for a Code Blue Medical Emergency, then call as per [RKPG Resuscitation: Code Blue Medical Emergency \(Acute\) Policy](#).⁵
- Contact the treating Consultant or Registrar/MO.

Discharge requirements

- The monitoring clinician performs one last visual check to ensure there are no signs or symptoms of PIS (if any concerns contact the nurse who administered the depot or if required the treating Consultant or Registrar) and then:
- Confirms patient is not travelling alone to their post-injection destination.
- Ensures patients have their patient card, understand when their next injection is due and are aware of symptoms of PIS to be vigilant of for the remainder of the day. Provide them with contact details for the service.
- Confirms the patient will not be driving or operating machinery for the remainder of the day.

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If the patient refuses to stay for the required period

- The monitoring clinician requests the patient to wait for at least the half hour of monitoring and to discuss leaving with the nurse who administered the depot. The treating Case Manager or Consultant or Registrar/MO will be contacted and attend as required.
- If a voluntary patient chooses to leave, the monitoring clinician should assess the patient to ensure they are not a risk to themselves or others.
- If the monitoring clinician believes the patient could be detained under the Mental Health Act 2014 they should arrange for an authorised mental health practitioner or medical practitioner to assess the patient.
- If the patient is assessed to have capacity to make the decision to leave, the patient can leave with a request to sign the observation sheet with the treating MO notified and a file note completed.
- Attempt to notify the identified primary carer immediately to inform them of the situation and precautions.
- A further attempt should be made to contact the patient/carers at two hours after administration of depot to ascertain that the patient has had not had symptoms of PIS.
- The treating MO/case manager should review the reason for the patient leaving to consider whether the administration of olanzapine pamoate is appropriate for that patient.
- Where a patient declines to give ongoing consent to participate in the monitoring requirements of olanzapine pamoate, other treatment options should be considered.
- If the patient continues to decline to give ongoing consent to participate in the monitoring requirements of olanzapine pamoate the decision to continue the olanzapine pamoate, despite non adherence to the monitoring requirements, is documented on the patient's medical record by the responsible Consultant Psychiatrist. The Consultant Psychiatrist also ensures that a Psolis alert summarizing this is generated which is printed out for the patient's medical record with a copy attached to the patient's depot chart.

Adverse events

- Any adverse event or instance where the patient leaves the community clinic before monitoring is completed will be documented in the medical record by the Clinician monitoring the patient as well as completing an incident form using the Datix Clinical Incident Management System.

Compliance monitoring

Documentation of monitoring requirements will be audited as per service audit standards.

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Links

[RKPG Hand Hygiene \(Infection Control\) Policy](#)

[RKPG Resuscitation: Code Blue Medical Emergency \(Acute\) Policy](#)

National Standards



National Mental Health Standards



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